

PROJECT 100 GUIDEBOOK

The condensed operating manual for a life rehabilitation comeback — derived from the Project 100 Encyclopedia V2.0, with forensic analysis for every protocol.



Bogdan — Project 100 training documentation. Personal photos integrated throughout this edition.

88.2 → 100 lb

Skeletal muscle mass target

22.9% → <15%

Body fat target

23.86 → 25.71

FFMI trajectory

V2.0

Derived from the Encyclopedia master

How to use this V2.0 — and what changed

This Guidebook is the short, runnable derivative of the **Project 100 Encyclopedia V2.0** (3 October 2026) — the single master volume that merged the Full Detailed Version and the Complete Clinical Analysis. Nothing in this Guidebook stands outside that master. It does not replace your locked doses or statuses; for nutrition, training, pills, supplements and vectors, each protocol carries **what it does, why it can help, where the risk actually lives, and how you monitor it**.

What stayed locked (Encyclopedia corrections): Zinc Supreme removed; separate copper paused; boron locked at 1 x 3 mg; V4 fixed at half the BioSteel serving; RapiDrem replaces BioSteel Hydration in V2 and V5; Canadian Carnivor credited with zero added creatine; V10 heating mandate retired; PUMP branch inactive. This Guidebook explains the locked record — it does not quietly change it.

THE V2.0 ANALYSIS PATTERN (USED IN EVERY CHAPTER)

Lens	Question it answers
Mechanism	How does this actually work in the body?
Stack interaction	What does it amplify or collide with elsewhere in the system?
Margin	How close are you to the known upper limit or symptom threshold?
Monitor	What do you watch, test, or log to stay safe?
Stop rule	What symptom or result pauses or removes it?

Important: This book documents your personal system. It is not medical advice and does not replace your clinician. Any change to prescription medication, or any new symptom, goes to a clinician first. Bring this book to appointments — especially the pill grid and lab pages.

Source of truth: Project 100 Encyclopedia V2.0 — 2026-10-03 (Full Detailed Version + Complete Clinical Analysis merged). Where a number appears below, it is the Encyclopedia number, not a new estimate. Known totals are partial label calculations, not a measured whole diet; whole-diet verification remains deferred, and the historical 2,780 kcal / 252 g protein / 69 g fat figures are legacy planning figures only.

The Prime Directive and the Return

PRIME DIRECTIVE

Rebuild to **100 lb skeletal muscle mass at under 15% body fat**, without trading long-term health for short-term numbers. Every protocol in this book must pass three locks: it must support training, it must be tolerable day after day, and it must leave a clean monitoring trail. If a protocol fails any lock, the protocol changes — not the target.

How the target is measured (Encyclopedia Ch. 1): 100 lb is a bioimpedance-estimated skeletal muscle mass, tracked on the same device under standardised conditions — waking, post-void, pre-intake, similar hydration and meal state. It is a personal target, not a clinical threshold. Creatine, electrolytes, carbohydrate and fluid shifts can move the estimate without new muscle, so quarterly review beats weekly reaction.

THE FIVE ERAS (2010 → 2026)

Era	What happened	What it taught the system
1. Foundation	Earlier athletic base	The body remembers; retraining is faster than first training
2. The Dark Stage	Detraining, life events, loss of structure	Without a system, drift is the default
3. The Excavation	Forensic audit of what remained	Start from measured truth, not memory
4. The Pivot (age 45)	Decision to rebuild deliberately	Age changes recovery math, not ambition
5. Project 100 (46)	Versioned, measured comeback	INTAKE x VECTOR x TIME, logged daily



Training documentation — the Pivot made visible.

The System: INTAKE x VECTOR x TIME

Everything in Project 100 reduces to one equation. **Intake** is what you take in (food, pills, supplements). **Vector** is the directed protocol that delivers it (V1–V10). **Time** is when it happens — and time is where most systems fail, because a good dose at the wrong hour becomes a bad dose.

	Training day	Rest day
Priority	Performance + recovery	Repair + tolerance reset
Active vectors	V1, V2, V3, V4, V10 — timed to the session	V3 (when consumed), V5 (if consumed), V10
Pill grids	Full training grids at lunch and afternoon	Training pill grids skipped
PUMP branch	Inactive on both branches — never combined with a morning stimulant session	
Nutrition	Higher energy, timed protein	Protein held, energy moderated
Monitoring	RPE, HR, session completion	Sleep, HRV, gut, mood

Analysis lens: Most supplement risk is not dose risk — it is *stacking at the same hour*. V2.0 therefore analyses every protocol twice: alone, and as stacked. The stacked analysis is the one that governs.

Training — Analysis



Progressive overload with joint protection — the training lock.

MECHANISM

Muscle is built by progressive mechanical tension plus sufficient protein and energy, and kept by repeating that signal before the previous adaptation fades. At 46, the synthesis side still works; the recovery side is the constraint. The program therefore buys growth with volume you can recover from, not volume you can survive once.

STACK INTERACTION

- **With stimulant vectors:** caffeine (IGNITER portion ~373 mg before tea or other sources; Earl Grey adds roughly 40–70 mg) raises output and masks fatigue — the mask is the risk. RPE must be logged honestly even on high-caffeine days.
- **With sleep:** a short night downgrades the next session automatically. No vector compensates for that downgrade.
- **With joints:** load progression is capped by joint feedback, not by target weights.
- **With hardware locks (Encyclopedia Ch. 3 & 6):** supported rows, presses and machine work dominate where spine, nerve, shoulder or elbow locks require protection; free axial loading is progressed conservatively and stopped by neurological signs.

MONITORING AND STOP RULES

Signal	Action
Joint pain that alters form	Stop that movement; substitute; log as symptom code
RPE drift up at same load, 2 sessions	Deload week; check sleep and intake before adding anything
HRV suppressed 3 mornings	Rest-day protocol until baseline returns

Nutrition — Analysis

MECHANISM

Protein drives muscle protein synthesis; total energy decides whether that synthesis can run; meal timing decides tolerance. Your system holds protein constant across training and rest days and moves energy with output — the reverse (cutting protein on rest days) is the most common comeback error, because repair peaks when you are not training.

STACK INTERACTION — THE GUT IS THE BOTTLENECK

- **No gallbladder context:** fat must be distributed across meals, not loaded. Large fat boluses plus magnesium plus vitamin C is the osmotic chain flagged in Encyclopedia Part IV — each is tolerable alone, together they produce urgent GI symptoms.
- **Legacy meal modules:** MS-01, SB-01 and FUSION_2A are preserved legacy records in Encyclopedia Ch. 5, printed with label-check macros. They are audit history, not a verified whole-day diet prescription.
- **Hydration:** electrolytes accompany training water, not as an afterthought; cramps are logged as data, not endured as noise.

Rule	Why (analysis)
Protein every meal, both day types	Synthesis signal must not blink
Fat spread, never bolused	Bile-independent digestion limit
Magnesium away from the largest meal	Breaks the osmotic stack
Caffeine cut-off respected	Protects the sleep that builds muscle



Recovery nutrition is a protocol, not a reward.

Pills — the 21+2 Grid, Analysed

The pill grid is the highest-trust page in the book: **23 audited records** (Encyclopedia Ch. 8), each with a status that was decided once and does not drift. This page adds the interaction analysis behind each status.

Status	Meaning	Analysis
Active	Take as scheduled	Re-checked against the full stack, not alone
Removed — Zinc Supreme	Do not take	System zinc already totals ~25 mg; removal prevents copper antagonism from worsening
Paused — Copper	Held pending review	Paused because zinc status governs copper need; recheck by lab, not by feeling
Locked — Boron 1 x 3 mg	Fixed dose	Locked low; boron shifts hormone binding, so the dose is a monitoring decision, not a growth lever

The rule that protects the grid: nothing enters the grid without a stacked analysis (mechanism + interaction + margin + monitor + stop rule). A good ingredient in the wrong stack is a bad protocol.

HIGHEST-ATTENTION INTERACTIONS IN THE GRID

- **Piperine-containing items** can inhibit CYP3A4 and P-glycoprotein, slowing clearance of many medications. Any prescription change must be checked against piperine timing.
- **Biotin (425 mcg total daily)** interferes with common lab assays (see Labs page) — the grid and the lab calendar are linked.
- **Piperine total:** about 14.5 mg on training days (9.5 + 5 mg) and 9.5 mg on rest days — the dose behind the CYP3A4 / P-glycoprotein caution above.

Supplements — Toxicology and Margins

Supplements are where enthusiasm usually outruns arithmetic. This page keeps the published totals beside their limits, exactly as Encyclopedia Part V does, with the analysis of what the margin means.

Item	Published system total	Analysis
Magnesium	341 mg baseline; 446–646 mg with optional Medi-C / Hey Relax	Supplemental scenarios sit at or above the 350 mg supplemental upper level; the high scenario plus vitamin C is the GI-risk stack
Vitamin C	452.9 → 2,452.9 mg range	High end approaches GI threshold; split and separate from magnesium
Zinc	~25 mg total	Within range only because Zinc Supreme was removed; do not re-add
Caffeine	IGNITER ~373 mg, before Earl Grey 40–70 mg or other sources	Training mornings regularly cross the 400 mg/day guidance when tea is summed; timing and sleep are the safety devices
Creatine HCl	1.5 g daily (training: V4 + V10; rest: V5 + V10)	Kidney monitoring by cystatin-C alongside creatinine, not creatinine alone (see Labs)

How to read a margin: a margin is not permission to spend it. It is the distance between your stack and the dose at which strangers in studies reported symptoms. Your personal threshold is found by logging, never by testing the edge deliberately.

Vectors V1–V10 — Analysis

A vector is a timed delivery bundle: several ingredients aimed at one outcome in one window (Encyclopedia Part III). Training branch: V1, V2, V3, V4, V10. Rest branch: V3, V5, V10. PUMP is inactive. Vectors are powerful because they stack — and every important risk in this book concentrates in the five cascades of Encyclopedia Part IV: **piperine, cholinergic, ALA–insulin autoimmune, osmotic, and stimulant**.

Vector family	Purpose	Stacking risk (analysis)	Monitor
Cognitive / nootropic	Focus, drive	Cholinergic overload: citicoline + alpha-GPC + DMAE + huperzine together can push acetylcholine too far — headache, GI, vivid dreams, low mood	Headache/mood log; never add a 5th cholinergic
Absorption enhancers	Bioavailability	Piperine: CYP3A4 / P-gp inhibition slows drug clearance	Medication list review at every change
Metabolic / insulin-adjacent	Glucose handling	ALA: rare Insulin Autoimmune Syndrome mechanism (HLA-associated) — know the hypoglycemia symptom pattern	Unexplained sweats/shakes = stop and seek care
Stimulant	Training output	IGNITER ~373 mg before tea; masks fatigue	Sleep latency; honest RPE
Recovery / GI	Tolerance	Magnesium + Vit C + fat bolus = osmotic chain	Separate in time; spread fat

Vector rule: when a symptom appears, remove the *newest or largest stacked vector first*, one change at a time, and log the result for 72 hours before changing anything else. Changing three things at once destroys the evidence.

Labs and Monitoring — Analysis

WHY YOUR LABS NEED CONTEXT

Two published cautions govern every blood draw in Project 100 (Encyclopedia Part VI). First, **biotin (425 mcg total daily) interferes with immunoassays**: depending on the assay, results can read falsely high or falsely low — classically troponin can read falsely low and several hormone assays can skew. There is **no universal 72-hour biotin hold**: disclose it and follow assay-specific instructions. Second, **creatine HCl (1.5 g daily) expands the creatinine pool**, so kidney checks should include **cystatin-C** alongside creatinine. March laboratory results predate the May supplement stack — do not read them as a verdict on the current stack. An afternoon testosterone result is non-diagnostic; hormone interpretation needs two fasting morning draws.

Before any draw	Do this
Biotin 425 mcg/day	Disclose to the lab; hold only per assay-specific instruction
Creatine HCl 1.5 g/day	Request cystatin-C alongside creatinine
Full stack	Bring the pill grid (page 7) printed
Symptoms	Bring the symptom-code log, dated

THE DAILY MONITORING MINIMUM

- Sleep duration and quality; morning HRV where available
- Session RPE and completion, honest on stimulant days
- GI tolerance (the osmotic stack announces itself here first)
- Mood and headache (the cholinergic stack announces itself here first)

Daily Execution — the Two Checklists

TRAINING DAY

- ☐ Prime Directive check: sleep adequate?
- ☐ Intake: protein anchored at every meal
- ☐ Vectors V1, V2, V3, V4, V10 timed; caffeine cut-off set; PUMP inactive
- ☐ Session: log load, RPE, joint feedback
- ☐ Recovery meal per Encyclopedia Ch. 5; fat spread
- ☐ Evening: magnesium separated from largest meal
- ☐ Log: symptoms coded; nothing changed mid-week without a note

REST DAY

- ☐ Protein held; energy moderated
- ☐ Rest vectors only: V3 when consumed, V5 if consumed, V10; training pill grids skipped
- ☐ HRV, mood, GI reviewed
- ☐ Grid statuses confirmed (no silent re-adds)
- ☐ Next session planned against recovery data
- ☐ Weekly: margins page re-read (page 8)

The rehabilitation rule: a rest day executed perfectly is training. The comeback is built on the days you do not lift.

Symptom Codes and the Failure Matrix

Symptoms are data with a code, a date, and a response — never a mood. Use the published codes from the Encyclopedia; the matrix below is the response discipline.

Pattern	Most likely stack	First response
Headache + vivid dreams + low mood	Cholinergic vectors	Remove newest cholinergic; 72-hour log
Urgent GI after meals	Mg + Vit C + fat bolus	Separate in time; spread fat; recheck scenario total
Sweats / shakes without cause	ALA / glucose vectors	Stop vector; seek clinical advice promptly
Poor sleep after training days	Caffeine timing	Move cut-off earlier; do not add sleep aids into the stack blindly
RPE drift, HRV suppressed	Total load	Deload; audit sleep and intake before supplements

Red flags — stop and seek clinical assessment (Encyclopedia Ch. 3 & Annex II): worsening or progressive weakness, bowel or bladder change, saddle numbness, jaundice, significant pain, recurrent watery stools, unexplained weight loss, fever with abdominal pain, chest symptoms, or persistently disrupted sleep. These are not training variables and never trigger dose escalation.

Failure rule: ambition never overrides a stop rule. A paused protocol is the system working, not failing.

REVIEW RHYTHM

- **Daily:** checklist + symptom log
- **Weekly:** margins page and HRV trend
- **Per lab cycle:** grid + biotin/cystatin-C protocol
- **Per version:** this book is re-issued; nothing is edited silently

Appendix — One-Page Reference

Item	Locked / Published
Target	100 lb skeletal muscle; <15% BF; FFMI 23.86 → 25.71
Equation	INTAKE x VECTOR x TIME; training day vs rest day
Zinc Supreme	Removed (system zinc ~25 mg)
Copper	Paused pending lab review
Boron	Locked 1 x 3 mg
Magnesium	341 / 446 / 646 mg scenarios; separate from Vit C and fat
Vitamin C	452.9 → 2,452.9 mg; split at high end
Caffeine	IGNITER ~373 mg before tea (Earl Grey +40–70 mg); timing protects sleep
Chronometer	Training: V1, V2, V3, V4, V10 + full grids. Rest: V3, V5, V10; grids skipped. PUMP inactive
V4 / RapiDrem	V4 = half BioSteel serving. RapiDrem replaces BioSteel Hydration in V2 and V5
Carnivor (V3)	Canadian panel: zero added creatine — do not credit 2.5 g
Creatine HCl / Biotin	1.5 g daily / 425 mcg daily — disclose both before labs; add cystatin-C
Nutrition totals	Partial label calculations only; 2,780 kcal / 252 g P / 69 g F are legacy planning figures
Labs	Disclose biotin 425 mcg + creatine HCl 1.5 g; cystatin-C with creatinine; no universal 72-h biotin hold
Vectors	Never stack a 5th cholinergic; piperine = medication check; ALA symptom pattern known

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